

Meridian Family Health Clinic

123 Elm Street, Suite 400 | Springfield, ST 00000 | Tel: (000) 555-0192 | SAMPLE DOCUMENT — NOT A REAL
PATIENT RECORD

CONSULTATION REPORT

Patient Name:	Jordan A. Whitfield	Date of Birth:	14-Mar-1990
Patient ID:	MRN-004821	Sex:	Male
Date of Visit:	01-Sep-2026	Referring Physician:	N/A (Self-referred)
Attending Physician:	Dr. Elena Marsh, MD (ENT)	Report Date:	01-Sep-2026

Chief Complaint

Facial pressure, nasal congestion, and purulent nasal discharge for 10 days, with associated headache and reduced sense of smell.

History of Present Illness

Patient reports onset of nasal congestion and clear rhinorrhea approximately 10 days ago, initially attributed to a common cold. Over the past 4–5 days, symptoms have progressed to include thick yellow-green nasal discharge, bilateral maxillary facial pressure that worsens on bending forward, frontal headache, mild fever (subjective, up to 100.8°F / 38.2°C two days ago), postnasal drip, sore throat, and intermittent cough worse at night. Patient denies visual changes, periorbital swelling, severe unilateral headache, neck stiffness, or altered mental status. No improvement with over-the-counter decongestants and saline rinses over the past 3 days. No prior episodes of similar severity in the past 12 months.

Past Medical History

Seasonal allergic rhinitis. No history of asthma, nasal polyps, immunodeficiency, or recurrent sinusitis. No prior sinus surgery.

Medications

Loratadine 10 mg PO once daily (as needed, seasonal). Oxymetazoline nasal spray, used for the past 4 days for symptomatic relief.

Allergies

No known drug allergies (NKDA).

Social History

Non-smoker. Occasional alcohol use. Works in an office environment. No recent air travel or known sick contacts with confirmed COVID-19 or influenza.

Review of Systems

General: Mild fatigue, low-grade subjective fever. **HEENT:** As above; denies ear pain, hearing changes, dental pain, or epistaxis. **Respiratory:** Mild nonproductive daytime cough, denies dyspnea or chest pain. **All other systems reviewed and negative.**

Physical Examination

Vitals: Temp 99.6°F (37.6°C), HR 82 bpm, BP 122/78 mmHg, RR 16/min, SpO2 98% on room air.

General: Alert, oriented, mild nasal congestion evident, no acute distress.

Head/Face: Tenderness to palpation over bilateral maxillary sinuses; mild tenderness over frontal sinuses. No periorbital erythema or swelling.

Nasal: Anterior rhinoscopy reveals erythematous, edematous nasal mucosa with purulent discharge in the middle meatus bilaterally. Nasal septum midline. No polyps visualized.

Oropharynx: Mild postnasal drip noted along posterior pharyngeal wall. No tonsillar exudate.

Ears: Tympanic membranes clear bilaterally, normal landmarks.

Neck: No cervical lymphadenopathy. No nuchal rigidity.

Lungs: Clear to auscultation bilaterally, no wheezes or crackles.

Assessment

Acute bacterial rhinosinusitis, bilateral (ICD-10: J01.90) — clinical diagnosis based on symptom duration exceeding 10 days without improvement, purulent nasal discharge, facial pressure/pain, and low-grade fever, consistent with published diagnostic criteria (e.g., IDSA/AAO-HNS guidance) for bacterial rather than viral etiology. No clinical signs of orbital or intracranial complication at this time.

Plan

- 1. Antibiotic therapy:** Amoxicillin-clavulanate 875 mg/125 mg PO twice daily for 7 days (first-line per current guidelines). Patient counseled on completing full course and reporting rash, GI intolerance, or lack of improvement.
- 2. Symptomatic management:** Intranasal corticosteroid (fluticasone propionate 50 mcg, 2 sprays each nostril daily) for mucosal inflammation. Saline nasal irrigation twice daily. Discontinue oxymetazoline immediately to avoid rebound congestion (rhinitis medicamentosa). Acetaminophen or ibuprofen as needed for pain/fever.
- 3. Supportive care:** Adequate hydration, rest, and use of a humidifier as tolerated.
- 4. Follow-up:** Return to clinic or seek urgent care if symptoms worsen, if fever exceeds 102°F, or if new visual changes, severe headache, facial swelling, or neurological symptoms develop (warning signs of possible orbital or intracranial extension). Routine follow-up in 2 weeks if symptoms persist or recur, with consideration of ENT referral and sinus CT imaging if no improvement on antibiotics or if recurrent episodes occur (≥ 3 –4 per year, suggestive of chronic sinusitis).
- 5. Patient education:** Discussed expected symptom course, red-flag symptoms, and appropriate antibiotic use provided.

Electronically signed by: Dr. Elena Marsh, MD

Date/Time: 01-Sep-2026, 3:42 PM

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